GOVCARE EHR

CKD AI CLINICAL DECISION SUPPORT REPORT

Clinical decision support only - clinician review required

PATIENT & REPORT DETAILS

Patient	Kamal Gunarathna
Patient number	PAT-2026-680166
DOB	2001-12-12
Sex	male
Assessment ID	bbb82bd3-14ef-4162-84d3-2d4559aad4d6
Assessment time	2026-10-10 10:12:42.463812
Hospital	National Hospital of Sri Lanka
Department	Out Patient Department
Clinician / operator	Local Super Admin
Model	GOVCARE-CKD-LOGISTIC-1.0

CKD RISK SUMMARY

Predicted CKD risk	Risk category	AI confidence
99.9%	**URGENT REVIEW**	**99.8%**

CKD CLASSIFICATION & CLINICAL CODING

Suggested CKD classification	Stage 3a
G category	G3a
Suggested ICD code	N18.31
Clinical description	Chronic kidney disease, stage 3a
Terminology system	http://hl7.org/fhir/sid/icd-10-cm
Basis	Reviewed eGFR 54 mL/min/1.73m²; chronicity and diagnosis require clinician confirmation
Coding status	AI_ASSISTED_SUGGESTION
Confirmation	Clinician confirmation required

A single eGFR result does not independently establish chronic kidney disease or chronicity. N18.6 (ESRD) is not inferred from eGFR alone.

○ G1	○ G2	● G3a	○ G3b	○ G4	○ G5

RENAL & ASSOCIATED FINDINGS

Parameter	Result	Unit	Source
eGFR	54	mL/min/1.73m²	PDF 01_full_ckd_text_report.pdf 95% confidence
Serum creatinine	1.4	mg/dL	PDF 01_full_ckd_text_report.pdf 95% confidence

Urine albumin/creatinine ratio (ACR)	42	mg/g	PDF 01_full_ckd_text_report.pdf 95% confidence
Urine protein	8	mg/dL	MANUAL
Systolic blood pressure	146	mmHg	PDF 01_full_ckd_text_report.pdf 95% confidence
Diastolic blood pressure	92	mmHg	PDF 01_full_ckd_text_report.pdf 95% confidence
HbA1c	7.2	%	PDF 01_full_ckd_text_report.pdf 95% confidence
BMI	27.4	kg/m²	PDF 01_full_ckd_text_report.pdf 95% confidence
Serum potassium	10	mmol/L	MANUAL
Hemoglobin	8	g/dL	MANUAL

RELATED DISEASE PROBABILITIES

Assessment-time model outputs. Disease probabilities are separate from AI confidence, measured abnormalities and confirmed diagnoses.

Disease / outcome	Probability	Time horizon	Model & availability
Current CKD model	**99.9%**	Current assessment	GOVCARE-CKD-LOGISTIC-1.0 · AVAILABLE Output of the existing CKD classifier. This is separate from model confidence and future disease risks.
CKD progression	**Data unavailable**	Not available	not-installed · NOT AVAILABLE No reviewed runtime adapter is installed for this outcome. No probability was calculated.
Kidney failure	**Data unavailable**	Not available	not-installed · NOT AVAILABLE No reviewed runtime adapter is installed for this outcome. No probability was calculated.
Acute kidney injury	**Data unavailable**	Not available	not-installed · NOT AVAILABLE No reviewed runtime adapter is installed for this outcome. No probability was calculated.
Cardiovascular disease	**Data unavailable**	Not available	not-installed · NOT AVAILABLE No reviewed runtime adapter is installed for this outcome. No probability was calculated.
Hyperkalemia	**Data unavailable**	Not available	not-installed · NOT AVAILABLE No reviewed runtime adapter is installed for this outcome. No probability was calculated.
Anemia	**Data unavailable**	Not available	not-installed · NOT AVAILABLE No reviewed runtime adapter is installed for this outcome. No probability was calculated.

Related-risk snapshot captured: 2026-10-10T04:42:42.467284800Z · Clinical interpretation required.

PREDICTIVE CLINICAL INTELLIGENCE - LONGITUDINAL REVIEW

Live authorized EHR history at report generation time. Projected values are statistical extrapolations and are never stored or presented as laboratory observations.

Recorded G / A category	Latest eGFR	Latest creatinine	Data completeness
- / -	**Unavailable**	**Unavailable**	**29% · LIMITED**

HISTORICAL eGFR & PROJECTED TRAJECTORY

No usable dated eGFR history is available.

Horizon	Current	Projected	Projected range	Trend	Reliability	Status / interval method
3 months	-	Unavailable	Not estimated	UNAVAILABLE	LOW	INSUFFICIENT DATA NOT ESTIMATED
6 months	-	Unavailable	Not estimated	UNAVAILABLE	LOW	INSUFFICIENT DATA NOT ESTIMATED
12 months	-	Unavailable	Not estimated	UNAVAILABLE	LOW	INSUFFICIENT DATA NOT ESTIMATED

FUTURE RISK / MONITORING STATUS

Module	Status	Category	Horizon	Method / version	Validation
CKD progression trajectory	INSUFFICIENT DATA	UNAVAILABLE	3 / 6 / 12 months	TREND PROJECTION trend-linear-2.0	STATISTICAL PROJECTION NOT CLINICALLY VALIDATED Local: NOT APPLICABLE
Kidney failure	NOT INSTALLED	-	2-year / 5-year	VALIDATED EQUATION not-installed	NOT INSTALLED Local: NOT LOCALLY VALIDATED
AKI safety	INSUFFICIENT DATA	INSUFFICIENT_DATA	Current data	RULE BASED govcare-longitudinal-2.0.0	CLINICAL RULE NOT PROBABILITY Local: NOT LOCALLY VALIDATED
Hyperkalemia monitoring	REVIEW REQUIRED	CRITICAL	Current data	RULE BASED govcare-potassium-1.0.0	CLINICAL RULE NOT PROBABILITY Local: NOT LOCALLY VALIDATED
Anemia review signal	REVIEW REQUIRED	HIGH_CONCERN	Current data	RULE BASED govcare-anemia-review-1.0.0	CLINICAL RULE NOT PROBABILITY Local: NOT LOCALLY VALIDATED
Cardiovascular risk	NOT INSTALLED	-	Model-specific	VALIDATED EQUATION not-installed	NOT INSTALLED Local: NOT LOCALLY VALIDATED

EXPLAINABILITY & DATA-QUALITY LIMITATIONS

Renal safety: INSUFFICIENT DATA. Renal observations have unresolved date, unit, provenance, plausibility or conflicting-result issues. Reconcile the source records before forecasting. A usable eGFR result from the previous 90 days is unavailable. A usable creatinine result from the previous 90 days is unavailable. At least two dated eGFR measurements spanning 90 days are required by the longitudinal sufficiency policy.

- MISSING CLINICAL DATE: A usable measurement date is required; upload time is never substituted.
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- MISSING REQUIRED METRIC: No usable egfr measurement is available.
- MISSING REQUIRED METRIC: No usable creatinine measurement is available.

Medication context: No recent active prescription record was found. Medication use and nephrotoxic exposure are unknown; reconcile with the patient.

CLINICIAN-CONTROLLED RECOMMENDATIONS

- No dated eGFR available | AWAITING REVIEW | Confirm whether a newer result exists and whether repeat measurement is clinically appropriate.
- No dated Urine ACR available | AWAITING REVIEW | Confirm whether a newer result exists and whether repeat measurement is clinically appropriate.

Method: TREND PROJECTION · Model: trend-linear-2.0 · Validation: STATISTICAL_PROJECTION_NOT_CLINICALLY_VALIDATED · Local validation: NOT_APPLICABLE

Clinical decision support only. Forecasts and risk estimates are not autonomous diagnoses.

CLINICAL DATA USED FOR ASSESSMENT

Parameter	Result	Unit	Source	Coding
Urine albumin/creatinine ratio (ACR)	42	mg/g	PDF 01_full_ckd_text_report.pdf 95% confidence	-
Age	24	-	CALCULATED	-
BMI	27.4	kg/m²	PDF 01_full_ckd_text_report.pdf 95% confidence	LOINC 39156-5
Blood urea nitrogen (BUN)	8	mg/dL	MANUAL	-
Diastolic blood pressure	92	mmHg	PDF 01_full_ckd_text_report.pdf 95% confidence	LOINC 8462-4
Edema	1	-	MANUAL	-
Family history of kidney disease	1	-	MANUAL	-
Fasting blood sugar	541	mg/dL	MANUAL	-
eGFR	54	mL/min/1.73m²	PDF 01_full_ckd_text_report.pdf 95% confidence	LOINC 48642-3
HbA1c	7.2	%	PDF 01_full_ckd_text_report.pdf 95% confidence	LOINC 4548-4
Hemoglobin	8	g/dL	MANUAL	-
NSAID use	1	-	MANUAL	-
Previous acute kidney injury	1	-	MANUAL	-
Urine protein	8	mg/dL	MANUAL	-
Serum creatinine	1.4	mg/dL	PDF 01_full_ckd_text_report.pdf 95% confidence	LOINC 2160-0
Serum potassium	10	mmol/L	MANUAL	-

Systolic blood pressure	146	mmHg	PDF 01_full_ckd_text_report.pdf 95% confidence	LOINC 8480-6

KEY FACTORS IN THIS MODEL OUTPUT

Contribution direction describes this model's prediction, not whether a clinical value is healthy, safe or protective.

Factor	Value	Unit	Direction	Contribution
Fasting blood sugar	541	mg/dL	Increases prediction	4.0812
Urine protein	8	mg/dL	Increases prediction	1.5332
Serum creatinine	1.4	mg/dL	Reduces prediction	-0.8219
Serum potassium	10	mmol/L	Increases prediction	0.7885
Edema	1	-	Increases prediction	0.5688
Blood urea nitrogen (BUN)	8	mg/dL	Reduces prediction	-0.5670
Hemoglobin	8	g/dL	Reduces prediction	-0.5232
Family history of kidney disease	1	-	Increases prediction	0.3341
eGFR	54	mL/min/1.73m²	Increases prediction	0.2926
Systolic blood pressure	146	mmHg	Increases prediction	0.1761
Urine albumin/creatinine ratio (ACR)	42	mg/g	Increases prediction	0.1474

RECOMMENDED CLINICAL REVIEW

1. Prompt clinician review is recommended.
2. Correlate with current renal function, urine findings, and clinical examination.

CLINICIAN REVIEW

Suggested classification	Stage 3a
Suggested code	N18.31
Coding decision	[ ] Confirm [ ] Modify [ ] Reject
Final clinician-selected code	_____
Clinical notes	_____ _____
Reviewed by / date	_____

This output does not diagnose disease or replace professional medical judgement. AI-assisted coding must be reviewed before a confirmed FHIR Condition is created.